



1111 3rd Ave. Suite 1100
Seattle, WA 98101

Date: 12-19-21

Radiantcare Radiation Oncology
4525 3rd Ave Se
Lacey, WA 98503

Re: New Commercial Fee Schedule

Dear: Radiantcare Radiation Oncology

Effective April 1, 2022, UnitedHealthcare will implement a new Commercial fee schedule for your practice.

Enclosed are specifications and a sample of the new fee schedule that outlines the reimbursement rates for Commercial benefit plans paid under the Payment Appendix - All Payer.

The new fee schedule will take effect 90 days after the date of this notification, per the terms of your current UnitedHealthcare agreement. **The attached fee schedule will replace the fee schedule currently associated with your UnitedHealthcare agreement for the Commercial benefit plans mentioned above.** It does not apply to Medicare Advantage Benefit Plans or Washington Medicaid and CHIP plans, and therefore is not replacing any applicable Medicare Advantage Benefit Plans or Washington Medicaid and CHIP plan fee schedule.

You are not required to sign or return anything to UnitedHealthcare. Please review the provisions of your agreement and regulatory appendix regarding any rights or obligations you may have related to this notice.

If you have any questions, please contact your contract manager, Lisa Moore at lisa.n.moore@uhc.com.

Thank you.

Sincerely,

A handwritten signature in black ink, appearing to read 'Scott Williams', written over a horizontal line.

Scott Williams

Vice President, Network Management

Enclosures
Amendment
Fee Schedule Sample Non-Facility
Fee Schedule Specification Sheet

Amendment to the Medical Group Participation Agreement

This amendment (this "Amendment") is to the Medical Group Participation Agreement (the "Agreement") between UnitedHealthcare Insurance Company, UnitedHealthcare of Washington, Inc., and PacifiCare Life and Health Insurance Company contracting on behalf of each entity individually, (collectively "Carrier") and Radiantcare Radiation Oncology ("Medical Group").

This Amendment is effective on April 1, 2022, subject to Section 4 below.

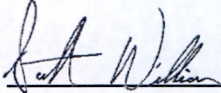
This Agreement is hereby amended as follows:

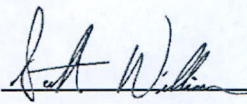
1. The capitalized terms used in this Amendment, but not otherwise defined, will have the meanings ascribed to them in the Agreement.
2. The payment appendix attached to the Agreement entitled, "Payment Appendix - All Payer" is deleted in its entirety.
3. The payment appendix attached to this Amendment entitled, "Payment Appendix - All Payer" is added to the Agreement.
4. Carrier may amend the Agreement by sending Medical Group a copy of the Amendment 90 days prior to the Amendment's effective date. Medical Group's signature is not required to make the Amendment effective. Medical Group may terminate the Agreement by providing Carrier 60 days prior written notice within 30 days of its receipt of this Amendment.

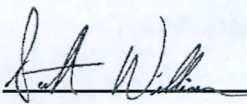
UnitedHealthcare
780 Shiloh Road
Plano, TX 75074

If we receive such notice from you during such time period, this Amendment will not take effect.

All other provisions of the Agreement shall remain in full force and effect.

UnitedHealthcare Insurance Company	The following is for identification purposes only:
Signature: 	Radiantcare Radiation Oncology
Print Name: <u>Scott Williams</u>	
Title: <u>Vice President, Network Contracting</u>	
Date: <u>12/202021</u>	

UnitedHealthcare of Washington, Inc.	The following is for identification purposes only:
Signature: <u></u>	Radiantcare Radiation Oncology
Print Name: <u>Scott Williams</u>	
Title: <u>Vice President, Network Contracting</u>	
Date: <u>12/20/2021</u>	

PacifiCare Life and Health Insurance Company	The following is for identification purposes only:
Signature: <u></u>	Radiantcare Radiation Oncology
Print Name: <u>Scott Williams</u>	
Title: <u>Vice President, Network Contracting</u>	
Date: <u>12/20/2021</u>	

For office use only:

Payment Appendix - All Payer

All Payer Fee Information Document: WA 95404/95405

Unless another Payment Appendix to this Agreement applies specifically to a particular Benefit Plan as it covers a particular Customer, the provisions of this Payment Appendix apply to Covered Services rendered by Medical Group to Customers covered by Benefit Plans sponsored, issued or administered by all Payers.

This Appendix applies to the following networks:

- ☒ Charter
- ☒ Choice
- ☒ Core
- ☒ Navigate
- ☒ Options
- ☒ Signature Value
- ☒ Select

The fee amounts under this Payment Appendix will change as further described in the attached Fee Information Documents.

Payment Appendix All Payer Fee Information Document

Representative Fee Schedule Sample for Hematology Oncology: as of 4/1/2022
Report Date: 12/14/2021

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Type Of Service Description	Primary Fee Source	Pricing Level
EVALUATION & MANAGEMENT	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
EVALUATION & MANAGEMENT - NEONATAL	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
EVALUATION & MANAGEMENT - PREVENTIVE	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
EVALUATION & MANAGEMENT - NURSING FACILITY SVCS	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - INTEGUMENTARY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - MUSCULOSKELETAL	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - RESPIRATORY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - HEMIC & LYMPHATIC	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - MEDIASTINUM & DIAPHRAGM	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - DIGESTIVE	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - URINARY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - MALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - FEMALE GENITAL	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - MATERNITY & DELIVERY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - ENDOCRINE	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - NERVOUS	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - EYE & OCULAR ADNEXA	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
SURGERY - AUDITORY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - BONE DENSITY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - CT	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - MAMMOGRAPHY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - MRI	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - MRA	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - NUCLEAR MEDICINE	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - PET SCANS	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - RADIATION THERAPY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
RADIOLOGY - ULTRASOUND	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
LAB - PATHOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	60.000%
OFFICE LAB	2020 CMS Clinical Lab Schedule National Limit	60.000%
CLINICAL LABORATORY	2020 CMS Clinical Lab Schedule National Limit	42.000%
MEDICINE - OPHTHALMOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
MEDICINE - CARDIOVASCULAR	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
MEDICINE - ALLERGY & CLINICAL IMMUNOLOGY	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
MEDICINE - CHIROPRACTIC MANIPULATIVE TREATMENT	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - PHYSICAL MED AND REHAB - MODALITIES	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - PHYSICAL MED AND REHAB - THERAPIES&OTHER	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - ENTERAL FORMULA	2020 UHC PEN NonRural Floor	60.000%
MEDICINE - OTHER	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
MEDICINE - IMMUNIZATION ADMINISTRATION	2020 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - CHEMO ADMIN	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
OBSTETRICS - GLOBAL	2020 CMS RBRVS Carrier Locality (0000000)	218.900%
IMMUNIZATIONS	UHC Immunization Fee Schedule	113.000%
INJECTABLES/OTHER DRUGS	CMS Drug Pricing	100.000%
INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - IVIG	CMS Drug Pricing	100.000%
INJECTABLES-SALINE & DEXTROSE SOLUTIONS	CMS Drug Pricing	100.000%
DME & SUPPLIES	2020 CMS DME NonRural Floor	60.000%
DME & SUPPLIES - RESPIRATORY	2020 CMS DME NonRural Floor	60.000%
DME & SUPPLIES - ORTHOTICS	2020 CMS DME NonRural Floor	60.000%
DME & SUPPLIES - PROSTHETICS	2020 CMS DME NonRural Floor	60.000%
DME & SUPPLIES - OSTOMY	2020 CMS DME NonRural Floor	60.000%
AMBULANCE	2020 CMS Ambulance Schedule - Urban (0000000)	218.900%

Default Percent of Eligible Charges: 40.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 79.00

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Last Routine Maintenance Update: 12-15-2021

Fixed Fees: 36415 - \$3.00 36416 - \$3.00 87804 - \$14.00 V5242 - \$2500.00 V5243 - \$2500.00 V5244 - \$2500.00 V5245 - \$2500.00 V5246 - \$2500.00 V5247 - \$2500.00 V5248 - \$5000.00 V5249 - \$5000.00 V5250 - \$5000.00 V5251 - \$5000.00 V5252 - \$5000.00 V5253 - \$5000.00 V5254 - \$2500.00 V5255 - \$2500.00 V5256 - \$2500.00 V5257 - \$2500.00 V5258 - \$5000.00 V5259 - \$5000.00 V5260 - \$5000.00 V5261 - \$5000.00 V5262 - \$2500.00 V5263 - \$5000.00

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

Section 1. Definition of Terms

Unless otherwise defined in this document, capitalized terms will have the meanings ascribed to them in the Agreement.

AMA: American Medical Association located at: www.ama-assn.org.

Anesthesia Conversion Factor: The dollar amount that will be used in the calculation of time-based and non-time based Anesthesia Management fees in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes. In the event that any of United's claims systems cannot administer a 15 minute anesthesia time unit value, then the Anesthesia Conversion Factor will be calculated as follows:

$$[(\text{Value of 15 minute Anesthesia Conversion Factor} / 15) * \text{anesthesia time unit value}]$$

For example, an Anesthesia Conversion Factor of \$60.00 (based on a 15-minute anesthesia time unit value) would be calculated to an Anesthesia Conversion Factor of \$40.00 (based on a 10-minute anesthesia time unit value).

$$\text{Example: } [(\$60.00 / 15) * 10 = \$40.00]$$

Anesthesia Management: The management of anesthesia services related to medical, surgical or scopic procedures, as described in the current Anesthesia Management Codes list attached to the Anesthesia Payment Policy located at www.unitedhealthcareonline.com.

Calculation of Anesthesia Partial Units:

Proration: Partial time units will be prorated and calculated to one decimal place rounded to the nearest tenth. For example, if the anesthesia time unit value is based on 15 minutes and if 17 minutes of actual time is submitted on a claim, then the 17 minutes will be divided by 15. The resulting figure of 1.1333 will be rounded to the nearest tenth and the total time units for the claim will be 1.1 time units.

In the event that any of United's claims systems cannot administer the calculation of partial units as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the Proration method described above.

CMS: Centers for Medicare and Medicaid Services located at: www.cms.hhs.gov.

CMS OPPSCap Rate: The Outpatient Prospective Payment System (OPPS) Cap Rate as defined in Section 5102(b) of the Deficit Reduction Act of 2005.

Conversion Factor: A multiplier, expressed in dollars per relative value unit, which converts relative values into Fee Basis amounts.

CPT/HCPCS: A set of codes that describe procedures and services, including supplies and materials, performed or provided by physicians and other health care professionals. Each procedure or service is identified with a 5 digit code. The use of CPT/HCPCS simplifies the reporting of services.

CPT/HCPCS Description: The descriptor associated with each CPT/HCPCS code.

Default Percent of Eligible Charges: In the event that a Fee Basis amount is not sourced by either a primary or alternate Fee Source, such as services submitted using unlisted, unclassified or miscellaneous codes, the codes are subject to correct coding review and will be priced at the contracted percentage indicated within this document.

Expired Code: An existing CPT or HCPCS code that will be expired by the entity that published the code (for example, CMS or the AMA).

Fee Amount: The contract rate for each CPT/HCPCS. The calculation of the Fee Amount is impacted by a variety of factors explained within this document including, but not limited to, Professional/Technical Modifier Pricing, Carrier Locality, CMS year, Place of Service and Pricing Level. The Fee Amount is calculated by multiplying the Fee Basis times the Pricing Level for each specific Type of Service.

Fee Basis: The amount published by the Fee Source upon which the Pricing Level will be applied to derive the Fee Amount.

Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule which supports the terms of the contractual agreement. This is the fee schedule for services performed in nonfacility Places of Service.

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: A 95405 - Facility

Fee Schedule Specifications: Documentation of the underlying calculation methodology and criteria used to derive the Fee Amounts contained within the fee schedule.

Fee Source: The primary or alternate entity or publication that is supplying the Fee Basis.

Fixed Fees: Fee Amounts that are set at amounts which do not change. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Flat Rate Fee: An amount published by a Fee Source and used as a Fee Basis that is other than a RVU, such as an amount for durable medical equipment or laboratory services.

Future Payment Terms: The general description of any pricing terms which will be implemented on a scheduled future effective date.

Last Routine Maintenance Update: The effective date on which this fee schedule was most recently updated. Please refer to the Routine Maintenance section of this document for more information about Routine Maintenance updates.

Linked Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule for each specific contractual agreement. This is the fee schedule for services performed in facility Places of Service.

Modifier: A Modifier provides the means to report or indicate that a service or procedure has been altered by some specific circumstance but not changed in its definition or code.

Place of Service: The facility or nonfacility setting in which the service is performed. This may also be referred to by CMS as Payment Type.

Pricing Level: The contracted percentage or amount that will be multiplied times the primary or alternate Fee Basis amount in order to derive the Fee Amount.

Primary Fee Source (Carrier Locality): The main Fee Source used to supply the Fee Basis amount for deriving the Fee Amount within each Type of Service category. For instance, if the Fee Amounts for a given category of codes are derived by applying a particular Pricing Level to the CMS Resource-Based Relative Value Scale (RBRVS), then CMS RBRVS is the Primary Fee Source. The Carrier Locality is designated to indicate the exact CMS geographic region upon which the Fee Amounts are based.

Professional/Technical Modifier Pricing: Fee Source-Based: Fee Amounts for Modifiers (for example, -TC or -26 Modifiers) are derived using the Fee Basis amount as published by the primary or alternate Fee Source.

RVU: Relative Value Unit as published by CMS. United uses the RVU that is used by CMS. For example, if CMS uses a transitional RVU, then United will as well.

Replacement Code: One or more new CPT or HCPCS codes that are the exact same services or descriptions and will replace one or more Expired Codes within the same Type of Service category.

Report Date: The actual date that this document was produced.

Representative Fee Schedule Sample: A representative listing of the most commonly used CPT/HCPCS codes and fees, along with other relevant pricing information, for each specific Fee Schedule ID. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Schedule Type: FFS: This is a fee-for-service fee schedule. Unless stated otherwise, the Fee Amount indicated will be used to calculate payment to you as further described within this document.

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 =F): This fee schedule follows CMS guidelines for determining when services are priced at the facility or nonfacility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule). CMS guidelines can be located at: www.cms.hhs.gov.

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

In the event that any of United's claims systems cannot administer the calculation of Site of Service Differential pricing as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the method described above.

Type of Service: A general categorization of related CPT/HCPCS codes. Type of Service categories are intended to closely align with the CPT groupings in the Current Procedural Terminology code book (as published by the AMA) and the HCPCS groupings (as published by CMS). The Office Lab Type of Service category represents those lab tests, as determined by United, in which the lab test result is necessary to make an informed treatment decision while the patient is in the office. A partial or complete crosswalk mapping of CPT/HCPCS to Type of Service categories is available to you upon request.

United: UnitedHealthcare Insurance Company or one of its affiliates which is a party to the Agreement.

Section 2. Alternate Fee Sources

In the event the Primary Fee Source contains no published Fee Basis amount alternate (or 'gap fill') Fee Sources may be used to supply the Fee Basis amount for deriving the Fee Amount. For example, if a new CPT/HCPCS code has been created within the Type of Service category of codes described above, and CMS has not yet established an RBRVS value for the code, we use Fee Sources that exist within the industry to fill that gap. For that CPT/HCPCS code, we adopt the RBRVS value established by the gap-fill Fee Source, and determine the Fee Amount for that CPT/HCPCS code by applying to the gap-fill RBRVS the same Conversion Factor and Pricing Level that we apply to the CMS RBRVS for those CPT/HCPCS code that have CMS RBRVS values. At such time in the future as CMS publishes its own RBRVS value for that CPT/HCPCS code, we would begin using the Primary Fee Source, CMS, to derive the Fee Amount for that code and no longer use the alternate Fee Source. Information about our Alternate and Primary Fee Sources can be located at www.unitedhealthcareonline.com >> Claims & Payments >> Fee Schedule Lookup >> Related Links.

Section 3. Routine Updates

Routine updates occur when United mechanically incorporates revised information created by the Fee Source, and as described below, to update the Fee Amounts calculated in accordance with this Fee Information Document. United routinely updates its fee schedule: (1) to stay current with applicable coding practices; (2) in response to price changes for immunizations and injectable medications; and (3) to remain in compliance with HIPAA requirements. United will not generally attempt to communicate routine updates of this nature.

The types of routine updates, and their respective effective dates, are described below.

a. Annual Changes to Relative Value Units, Conversion Factors, or Flat Rate Fees

This fee schedule follows a "stated year" construction methodology. The 2020 RVU, the 2020 Conversion Factor, and the 2020 Flat Rate Fee will be locked in as the basis for deriving Fee Amounts.

Generally, any RVU, Conversion Factor, or Flat Rate Fee changes published in subsequent years by the Primary Fee Sources will not be reflected in this fee schedule except, for example, to add Fee Amounts for new codes or to replace alternate Fee Basis amounts. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

In the event that a code contains a status code of "C" (indicating the code is carrier priced), United will establish Fee Amounts using the following methodology:

- 1) If CMS' multiple procedure indicator is other than "4" and United's multiple imaging reductions do not apply, United will establish Fee Amounts for those codes and modifiers using the CMS OPPSCap Rate, if available.
- 2) In all other cases (including if a CMS OPPSCap Rate is not available), United will use reasonable commercial efforts to establish Fee Amounts for all modifiers associated with the code based on fee information available and published by (in order of preference) CMS, the local fiscal intermediary or fiscal intermediaries from other locations.

b. Quarterly Updates in Response to Changes Published by Primary and Alternate Fee Sources

United updates its fee schedule in response to changes published by Primary Fee Sources as a result of additions, deletions, and changes to CPT codes by the AMA or HCPCS codes by CMS and any subsequent changes to CMS' annual update. United updates its fee schedules for new CPT/HCPCS codes using the applicable Conversion Factor and Pricing Level of the original construction methodology along with the then-current RVU of the published CPT/HCPCS code. The effective date of the updates described in this subsection b. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection b. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

update under this subsection b. will be effective no later than October 1.

In the event that a code contains a status code of "C" (indicating the code is carrier priced), United will establish Fee Amounts using the following methodology:

- 1) If CMS' multiple procedure indicator is other than "4" and United's multiple imaging reductions do not apply, United will establish Fee Amounts for those codes and modifiers using the CMS OPPSCap Rate, if available.
- 2) In all other cases (including if a CMS OPPSCap Rate is not available), United will use reasonable commercial efforts to establish Fee Amounts for all modifiers associated with the code based on fee information available and published by (in order of preference) CMS, the local fiscal intermediary or fiscal intermediaries from other locations.

However, in the event that the code source has expired a CPT/HCPSC code and replaced it with a Replacement Code, United will crosswalk the fee from the Expired Code to its Replacement Code as further described below:

Based on information published by the code source (AMA Current Procedural Terminology and The HCPSC Level II), when one Expired Code is replaced by one Replacement Code, United will apply the Expired Code's Fee Amount to the Replacement Code; provided, however, if the Expired Code's Fee Amount was determined by an alternate Fee Source and a Primary Fee Source becomes available, the Replacement Code's Fee Amount will be determined using the Primary Fee Source.

Based on information published by the code source (AMA Current Procedural Terminology and The HCPSC Level II) and United's claims data, when several Expired Codes that are always done in conjunction with each other are replaced by one Replacement Code, United will apply the sum of these Expired Code's Fee Amounts to the Replacement Code; provided, however, if the Expired Code's Fee Amount was determined by an alternate Fee Source and a Primary Fee Source becomes available, the Replacement Code's Fee Amount will be determined using the Primary Fee Source.

The following types of codes are not included in our direct crosswalk methodology as described above:

- Temporary HCPSC codes, such as G, K, Q, and S codes
- Temporary CPT codes, such as Category III codes
- Informational codes, such as CPT Category II codes
- HCPSC-C Codes, which are only used by hospitals
- Codes categorized as immunizations and injectables

If any types of codes not currently listed in the exclusions above are developed in the future, United reserves the right to make a crosswalk determination at that time.

c. Price Changes for Immunizations and Injectables

United routinely updates the Fee Amounts in response to price changes for immunizations and injectables published by the Fee Sources. In addition, United's Executive Drug Pricing Forum (EDPF) meets on a quarterly basis to review and evaluate the drug prices that will be used in each quarterly update. The EDPF may address topics including pricing for emerging drugs, anticipated manufacturer price changes, and special circumstances (for example, H1N1 vaccine). Based on supporting information provided by the drug manufacturer or the Fee Source, United's EDPF may elect to establish a Fee Amount or override a Fee Amount, as published by the Fee Source, in favor of a Fee Amount that is more appropriate and reasonable for a particular vaccine or drug. These Fee Amount updates will be effective as described below.

For Immunizations, United applies the UHC Immunization Fee Schedule. The Centers for Disease Control and Prevention Private Sector Selling Price (CDC PSSP) is the Primary Fee Source used to obtain the Fee Basis amounts. In the event that more than one Fee Basis amount is published by the CDC PSSP for a specific CPT/HCPSC code, an average of the published amounts will be used.

More information about the UHC Immunization Fee Schedule can be located at: www.unitedhealthcareonline.com >> Claims & Payments > Fee Schedule Lookup > Related Links "UHC Immunization Fee Schedule"

The effective date of updates under this subsection c. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection c. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection c. will be effective no later than October 1.

d. Other Updates

Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA 95404 - NonFacility
Linked Fee Schedule ID: WA 95405 - Facility

United reserves the right, but not the obligation, to perform other updates as may be necessary to remain consistent with a Primary Fee Source. United also will perform other updates as may be required by applicable law from time to time. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

Section 4. Miscellaneous

Claims must be submitted using a CMS 1500, its successor form or its electronic equivalent. All claims submitted under this Appendix must use CPT Codes, HCPCS Codes, ICD-9 codes or its successor and other codes in compliance with HIPAA standard data set requirements. Claims submitted without HIPAA standard data set requirements may be denied.

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers.) As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies.

No payments will be made for any CMS additional compensation programs under this Payment Appendix, including without limitation value based modifiers, incentive programs or other bonus payment programs.

Section 5. Services Covered or Provided by Another Program

If an applicable state, federal or other program is available to provide items or payment directly to provider for specific covered services for customers subject to this Appendix that would otherwise be payable under this Appendix, the applicable program will apply and not this Appendix. (For example, the Vaccines For Children program currently provides vaccines free of charge, and therefore no amount will be payable under this Appendix for vaccines within the Vaccines For Children program.)

For More Information United is committed to providing transparency related to our fee schedules. If you have questions about this fee schedule, please contact Network Management at the address and phone number on your contract or participation agreement. Alternatively, you may use our fee schedule look-up function on the web at: www.unitedhealthcareonline.com or contact our Voice Enabled Telephonic Self Service line at (877) 842-3210.